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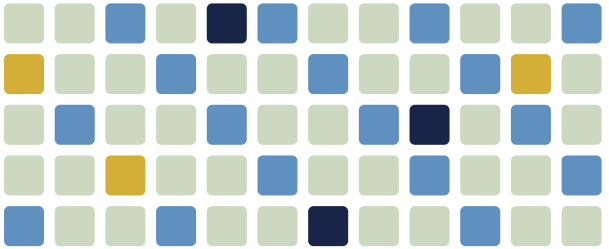
For the attention of Mrs Bukola Ladipo

HR AND WORKFORCE SERVICES / 23 SEPTEMBER 2026

Finding, assessing and keeping **the people a hospital runs on**

The healthcare HR and workforce practice at Consult for Africa. Recruitment and executive search, leadership assessment and coaching, culture measurement, workforce planning, and interim leadership.

Prepared after our meeting, in answer to your question about what HR support we can offer the Hospital.



Dr Debo Odulana, Founding Partner
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You asked what we can do on the people side

- **You raised it, so this answers it directly.** A short prospectus of the HR and workforce practice, what each service is, how it runs, and what it costs.
- **It is not a new subject between us.** When we first wrote to you in March, difficulty with hiring was one of the four challenges set out at the top of our proposal. It has been in the frame from the beginning.
- **And it connects to the risk work.** Establishment against the rota, key person dependency in scarce specialties, and whether people feel able to speak up are all workforce questions that land on an enterprise risk register.

A complimentary place on the leadership assessment is yours if you would like to see it from the inside before you decide anything. We will send you a personal link.

Everyone is hiring from the same shrinking pool

None of this is news to an HR manager in Lagos. It is here because it explains why the answer is rarely another advert, and why the practice is built the way it is.

80%
of Nigerian healthcare workers say they intend to emigrate

16,000
doctors left in five years, against roughly 55,000 licensed

8,560
registered with the UK regulator between 2021 and 2024

16
professional cadres, not one. Nursing and allied staff decide as much as doctors do

> The consequence for a hundred bed hospital is not abstract. It is that the consultant you need has three offers, the nurse you trained is being recruited from abroad, and the cost of a senior hire who does not work out is measured in years rather than months.

OUR THESIS

Culture is the multiplier, not the add-on

When something goes wrong in a hospital, the presenting cause is almost always a thing. A missing drug, a generator nobody tested, a form that was not completed. The thing is real and it should be fixed. But the reason it happened is almost never the thing, and fixing it does not stop the next one.

Three things make a culture, and none of them is a poster

The routines people follow

Not the procedure in the binder, the habit at three in the morning when nobody senior is on the floor. Routines are established deliberately or they are not established at all.

The incentives you actually pay

Job descriptions, targets and commission structures tell people what you value far more loudly than a values statement does. Where the two disagree, the incentive wins.

Whether it is safe to say something

People report when they believe nothing happens to them for reporting. Everything you can measure about safety follows from that one belief.

The missing item is the symptom. The absent routine and the unaligned incentive are the disease. We have said that to a board before, and it is why recruitment, assessment, coaching and pay are sold here as one practice rather than four products.

Six services, and they are usually bought in this order

Service	What it is	When a hospital reaches for it
Recruitment and executive search	Market mapping and direct approach for clinical and executive roles, including senior Nigerians abroad where the role suits a returner.	A vacancy that has been open too long, or a role too senior to advertise.
Leadership assessment	A structured six dimension assessment of how somebody leads, used at hire or to develop the people already here.	Before promoting a clinician into management, or after a senior hire has not worked.
Coaching and development	One to one and group coaching with certified African coaches, built on what the assessment surfaced.	A capable clinician struggling in a leadership role, which is the most common and most fixable problem we see.
Culture and engagement measurement	Anonymous staff instruments, reported in aggregate, that tell you what people will not say in a meeting.	Attrition you cannot explain, or a board that wants evidence rather than impression.
Salary benchmarking and workforce planning	What roles actually clear at, banded by cadre and seniority and loaded with employer on costs. Establishment against actual service, and rota analysis.	Budgeting, a counter offer, a new service line, or a suspicion that the establishment on paper is not the one on the floor.
Interim and fractional leadership	A senior operator in post part time, or a full time interim while you search.	A gap you cannot leave open and cannot fill quickly.

ONE OF SIX

Recruitment and executive search

- **We map the market rather than advertise into it.** For scarce clinical roles the person you want is employed, not looking. The work is identifying them and making the approach, which is why we do not run this as a job board.
- **A bank of more than ten thousand professional records.** Across all sixteen cadres, built from the professional registers, with specialty, location and experience held against each. It is a starting point for a search rather than a shortlist.
- **Senior Nigerians abroad, where the role suits a returner.** Through the Doctors Foundation for Care, a network of more than five hundred diaspora physicians. Most will not relocate, and the ones who might are worth knowing.
- **Assessed before you meet them, if you want that.** Any shortlisted candidate can carry a structured leadership assessment, so you are choosing on evidence rather than on interview performance.

HOW IT IS STRUCTURED

Fee
8 to 15 per cent of first year gross salary. The lower end for nursing and allied batches, the upper for a scarce senior specialist

Delivery
six to ten weeks to shortlist

Guarantee
three to six month free replacement

Payment
staged, not all on offer

Exclusivity
not required, though it changes what we can commit to

Leadership assessment

Maarova is our own assessment platform, built for healthcare and calibrated to the African context rather than translated into it. It measures how somebody leads, not whether they are clever.

Behavioural style

how they work, and how that reads to a team

Values and drivers

what they optimise for when it is difficult

Emotional intelligence

self awareness, regulation, empathy

Clinical to leadership

the transition that breaks good clinicians

Team and culture

how they shape the unit around them

360 feedback

how the people around them actually experience it

- **Three streams, and you pick one per cohort.** Recruitment, to decide on a hire. Development, to build the people you have. Organisational intelligence, to see the whole leadership landscape at once.
- **The report lands within minutes, and points to a person.** Every output routes to a coach rather than to a score. The platform supports the relationship, it does not replace it.

Take it yourself, at no charge. It is about twenty minutes, the report is yours to keep, and it is the fastest way to judge whether it is worth putting in front of your leadership team.

Coaching and leadership development

The most common workforce problem in a private hospital is not a bad hire. It is a very good clinician who has been promoted into management and given no support, and who is now quietly struggling while everyone assumes competence in one thing transfers to the other.

Format	Shape	Best for
One to one coaching	Twelve sessions over six months, biweekly, video with messaging between.	A named individual in a role that matters, usually after an assessment.
Group coaching	Four to six peer leaders, monthly, over three to six months.	A layer of middle managers who are all hitting the same wall.
On site workshop	Half or full day, ten to thirty leaders.	Setting a shared language quickly, often as a programme opener.
Leadership retreat	Two to three days offsite, ten to twenty leaders.	A leadership team that needs to work on itself rather than on the business.

Coaches are certified and matched rather than allocated, with a chemistry call before anything starts and one free switch inside the first two sessions. The evidence is consistent that the quality of that relationship predicts the outcome more than the credentials or the method do, so we treat the match as the intervention.

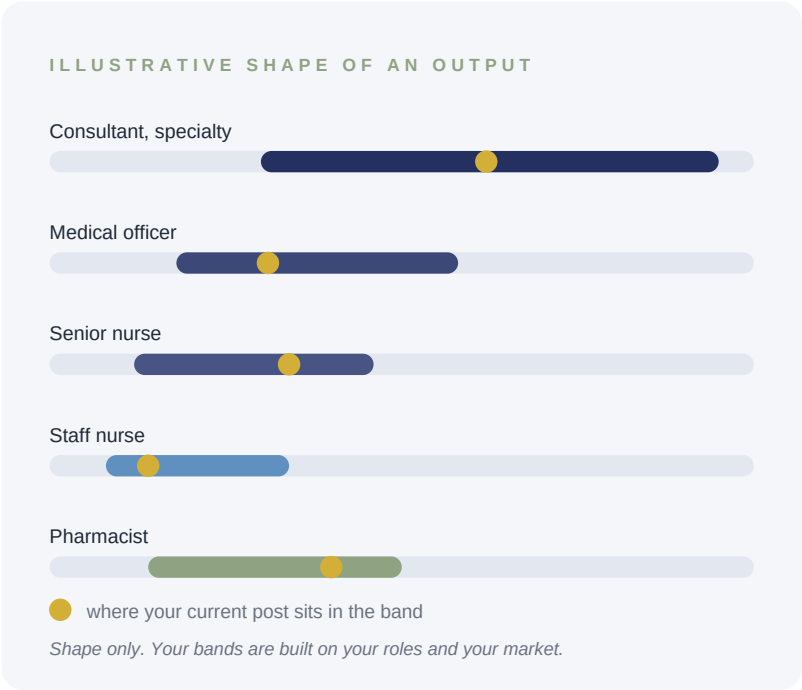
Culture and engagement measurement

- **Anonymous, and reported only in aggregate.** Staff answer honestly when they believe that, and not before. Your chief executive says so in writing before the first form goes out, and we never report a unit small enough to identify anyone.
- **Adapted from a validated hospital instrument, with local items added.** Not a generic engagement survey. It asks about speaking up, about whether mistakes are treated as learning, about handover and about whether people feel able to stop the line.
- **Deployed as a mobile form with printed codes, so response rates hold.** Paper and web generated from one definition so the two cannot drift, and results land in a live view rather than a spreadsheet months later.
- **It gives the board evidence rather than impression.** Attrition is explained after the fact by whoever is asked. A measured culture baseline lets you see it building, and see whether an intervention moved it.

What a role actually clears at, not what it was advertised at

Pay decisions in this market are too often made off last year's budget and a rumour about what a competitor pays. That is how a hospital loses a nurse it trained and overpays for one it did not need.

- **Three sources, not one.** Salary reported by professionals themselves on our workforce platform, cadre by cadre and city by city. The public sector scales, which anchor the floor for much of the clinical workforce. And our own search data, which is what roles actually closed at rather than what was posted.
- **Banded by cadre and seniority, not averaged.** An average is useless when the spread is the whole point. You get a floor, a midpoint and a ceiling for each role, and where your current post sits inside it.
- **Loaded properly, so the number is the real one.** In Nigeria employer on costs add roughly a fifth to a gross salary once pension, the thirteenth month, statutory contributions, group life and medical cover are counted. Budgets routinely miss it, which is why a post that looked affordable is not.



> Most often used at budget, when a service line opens, when a counter offer is on the table, or when a board asks why attrition is what it is and nobody can answer with a number.

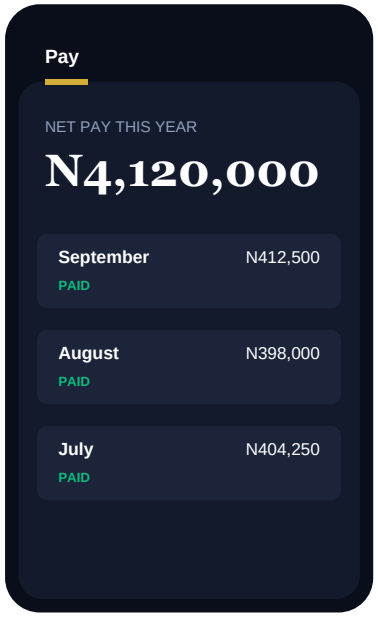
Workforce planning, pay, and interim leadership

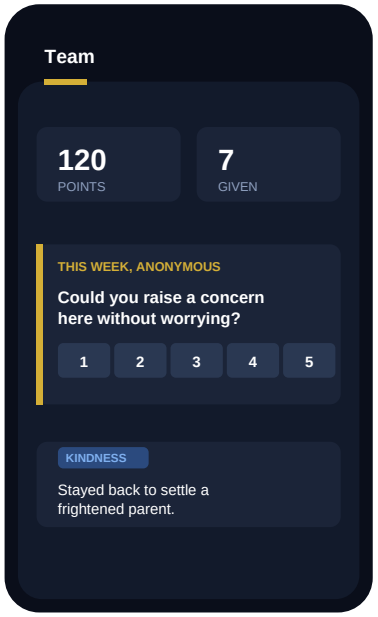
The least glamorous of the six and often the one that finds the most money.

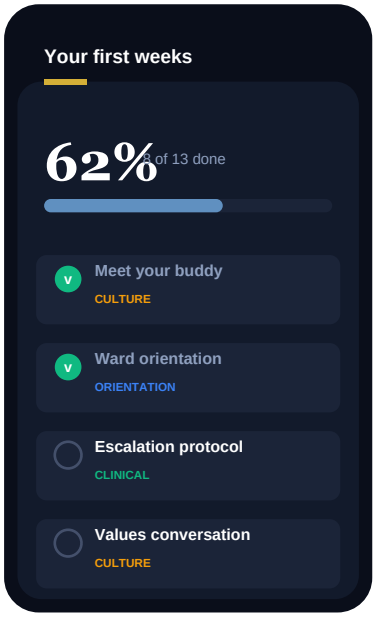
- **Establishment against actual service.** We count from the rota rather than the organisation chart. On a recent hospital engagement the reports showed six registered nurses covering a period when the rota showed four, and nobody had reconciled the two.
- **Grading, pay and the true cost of a post.** Benchmarked against real market data rather than last year's budget, and loaded properly. In Nigeria the employer on costs add roughly a fifth to a gross salary once pension, the thirteenth month, statutory contributions, group life and medical cover are counted, and budgets routinely miss it.
- **Interim and fractional leadership.** A senior operator in post part time, or full time while you search. We have placed and we have taken the seat ourselves, which is a different level of accountability from recommending someone.

Pay, recognition and belonging, in their pocket

HospitlOS is our hospital operating layer. The worker app is the half your staff hold. It is where the rota, what they have been paid, whether anyone noticed their shift, and how their first weeks are going stop being HR processes and become something a nurse can check on a Sunday night.







WHAT IT COVERS

- Payroll and payslips, issued only, never drafts
- Recognition against your stated values, visible to the team
- Weekly pulse, anonymous by design
- Onboarding and cultural integration, day by day
- Shift posting, claiming and assignment
- Availability set by the person, not the roster clerk
- Handover captured at the shift, not after it
- Leave, training records and appraisals

The pulse carries no staff identifier at all. A one way hash stops a second response without making the first one attributable, because people answer honestly when that is true and not before. That is the culture thesis on slide four, written into the data model rather than into a policy.

What each of these costs, indicatively

Ranges rather than quotes, because the number turns on cohort size, seniority and site. Anything here is scoped properly before it is priced.

Service	Basis	Indicative
Recruitment and executive search	Percentage of first year gross salary, tiered by seniority and volume	8 to 15 per cent, with a three to six month replacement guarantee
Leadership assessment	Per leader assessed	N460,000 to N690,000
Assessment with group coaching	Per leader, six monthly sessions	N920,000 to N1,380,000
Assessment with one to one coaching	Per leader, twelve sessions over six months	N2,300,000 to N4,025,000
Leadership retreat	Per participant, two to three days	N1,725,000 to N2,875,000
Culture and engagement measurement	Per site, per cycle	Scoped on the size of the staff body
Interim and fractional leadership	Monthly retainer against agreed days	Scoped on the role

WHERE WE HAVE DONE THIS



The people work, not the slideware

- **Havana Specialist Hospital, Lagos.** A governance capability programme delivered across three cohorts, closing with a simulated board meeting and a per person competency check. Our founding partner chairs its Audit and Risk Committee.
- **Haven Paediatric Centre, Lagos.** An establishment and culture engagement. Anonymous staff safety culture instrument, a patient experience instrument, and a rota level review that found the staffing on paper was not the staffing on the floor.
- **Medbury Healthcare Group.** Senior recruitment mandates at business unit lead and chief operating officer level, run as search rather than advertising.
- **House of Refuge, Lekki.** Leadership assessment across a staff body, with the coach trained on what it surfaced rather than on a standard curriculum.

References are available on request. Tell us which of these you would like to speak to and we will make the introduction, with their consent.

Oludayo Olugbamigbe, HR Practice Lead

SPHRi, HR Certification Institute
Associate, Chartered Institute of Personnel Management
MSc Industrial Relations and Personnel Management, Distinction, University of Lagos

Oludayo joins the practice from the Nigerian Exchange Group, where she is Group Talent Management Specialist and HR Business Partner across the group's entities. Her work there is the unglamorous end of people strategy done properly: a talent identification and assessment framework, a two line succession framework for critical roles, and an employee value proposition rebuilt around why people were leaving.

A psychologist before she was an HR practitioner

Which is why assessment, coaching and culture measurement sit naturally in this practice rather than being bought in.

WHAT THAT LOOKS LIKE IN NUMBERS, AT NGX

+25%
leadership pipeline readiness, through a structured succession framework

95%
of the workforce classified by her talent identification and assessment framework

5%
organisational attrition, down through a rebuilt employee value proposition

+65%
application completion, after she reworked the group careers portal

She leads the practice day to day. Dr Debo Odulana holds partner oversight, and the assessment and coaching network sits behind both of them. For a hospital that means one person accountable for the people work, rather than a rotating cast of specialists.

WHAT WE SUGGEST

Two small things, neither of which commits you

You asked what we could offer. The honest answer is that it depends on which of the six is actually your problem, and we would rather find that out than sell you a programme.

01

Take the assessment yourself

At no charge. About twenty minutes, the report is yours, and you will know within a day whether it is something you would put in front of your consultants and unit managers.

02

Half an hour on the two roles that are hardest to fill

Tell us which posts have been open longest and we will tell you plainly whether it is a search problem, a pay problem or a proposition problem. That conversation costs nothing and is usually the most useful thing we do first.

A personal link for the assessment follows this document. If it is easier, reply to this email and we will set it up for you and for anyone else on your team you would like included.

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The building is the easy part.
It is the people who decide.

Every hospital we are called into has the same three problems underneath the presenting one: the right people are hard to find, the ones who are promoted are rarely supported, and nobody is measuring whether the culture can carry the ambition. That is the practice, and that is what we would like to talk to you about.



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